

Agentic AI Against Aging Hackathon: Ovarian Aging Value Report

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1. Executive Summary

This report estimates the economic value of the 5-year therapeutic menopause delay for women in the US who enter it before 45. It was completed as part of the Agentic AI Against Aging Hackathon. First, we present our AI pipeline that collects, grades, and extracts papers with their relevant metrics. Second, we document our economic methodology for estimating annual US direct and indirect costs and total addressable market. Third, we conclude the report by presenting the results.

The AI pipeline autonomously discovers and extracts risk metrics from research papers linking menopause timing to health outcomes across eight disease categories. Using a LangGraph-based state machine, the system iteratively generates PubMed queries, evaluates abstract relevance, downloads open-access PDFs, and extracts structured risk metrics (OR, HR, RR) with confidence intervals while assessing study quality via ROBIS criteria (Whiting et al., 2016). The fully autonomous pipeline processed 1655 meta-analyses and extracted 584 high-quality risk metrics with complete metadata traceability.

In the economic methodology, we first calculate the net effect therapeutic menopause delay has on annual incidents per 100,000 women for: all-cause mortality, type 2 diabetes, cardiovascular disease, all-cause dementia, osteoporosis and fractures, breast cancer, endometrial cancer, and ovarian cancer. Then, we estimate the annual direct and indirect cost per incident for each disease. Multiplying the annual cost per incident by the net change in incidents per 100,000 women gives us the total yearly cost. A 5-year therapeutic delay would save these costs 5 years in a row; hence, we calculate the total present value of these savings by applying a 3% discount rate (4.5797 multiplier).

Lastly, we scaled these results to the appropriate US population. 1.3 million women enter menopause in the US every year (Peacock et al., 2023). Of these, 10% have it before 45 years old, which totals 130,000 women (Anagnostis et al., 2025). Scaling our costs for 100,000 women to 130,000 gives the yearly addressable market in the US. The total addressable market applies these costs to 10% of all women in the US: 17,177,300.

Due to the scarcity of data, the chose cohort to study were women who enter menopause before and after 45. Of notable health impacts RR for all-cause mortality was 1.12, and for cardiovascular disease 1.14. This would result in 90 deaths per 100,000 people yearly, with 98 new cardiovascular cases. When scaled to 130,000 women with application of a 5-year multiple, the yearly total costs are \$429,601,314. Of which \$83,943,000 are direct, and \$345,658,000 are indirect. When scaled to 17,177,300 women, this results in a \$56,707,373,000 total addressable market. Due to time and data constraints, the final costs are first and foremost *rough* estimates, and we recommend considering them within a $\pm 33\%$ range.

George Box famously wrote “All models are wrong, but some are useful”; we hope our report will be in line with that spirit.

2. AI Pipeline

Source code available at:

<https://github.com/EliasSchlie/quantifying-the-value-of-delayed-ovarian-aging>.

2.1 Overview and Motivation

Systematic literature reviews require substantial manual effort: searching databases, screening hundreds of abstracts, downloading papers, extracting data tables, and assessing study quality. For our menopause timing research, we needed to collect risk metrics across eight disease categories from meta-analyses—a task that would traditionally require weeks of manual work by trained researchers.

We developed an autonomous AI agent to automate this entire pipeline, from query generation to structured data extraction. The system operates as a LangGraph state machine with nine nodes that iteratively discover relevant papers, evaluate their quality, and extract risk metrics with full metadata traceability. This approach enabled us to process 1655 papers and extract 584 metrics in a fraction of the time required for manual review, while maintaining rigorous quality standards through automated ROBIS assessment.

2.2 Architecture

The agent is implemented as a directed graph workflow with three types of nodes: AI-powered decision nodes, rule-based action nodes, and conditional routing nodes (see Figure 1 for the complete workflow diagram). The workflow begins with query generation and terminates when either the target number of metrics is reached or the search space is exhausted.

AI-Powered Nodes:

- **create_query**: Generates targeted PubMed search queries using domain knowledge, adapting based on previous attempts
- **check_abstract**: Evaluates abstract relevance using LLM classification (relevant/not relevant)
- **extract_metadata**: Extracts study metadata including sample size, geography, confounders, and number of included studies
- **evaluate_robis**: Assesses study quality using the ROBIS framework, providing both categorical risk rating (Low/High/Unclear) and numerical validity score (0-10)

- **extract_risk_metrics:** Uses structured tool calls to extract odds ratios (OR), hazard ratios (HR), and relative risks (RR) with 95% confidence intervals

Deterministic Nodes:

- **search_pubmed:** Queries PubMed E-utilities API for meta-analyses (up to 500 results per query)
- **filter_papers:** Deduplicates papers based on DOIs of already-checked publications
- **download_paper:** Downloads PDFs via DOI resolution with multiple fallback strategies, then converts to markdown
- **track_failed_download:** Logs paywalled/closed-access papers to CSV for subsequent manual access through institutional subscriptions and processing via the paywalled content pipeline

The workflow incorporates four conditional routing points that determine next actions based on state (see Figure 1).

2.3 Key Technical Components

PubMed Integration: The system queries the NCBI E-utilities API with programmatically generated search strings that combine menopause timing terminology with disease-specific keywords. Queries are filtered to meta-analyses only, and the agent learns from unsuccessful queries to reformulate searches.

PDF Acquisition: PDF downloads employ a multi-strategy approach: (1) direct arXiv access for preprints, (2) Unpaywall API queries for open-access versions, (3) Bright Data web unlocker for publisher sites, (4) direct DOI resolution, and (5) HTML-to-PDF conversion as a last resort. Failed downloads are logged for manual processing.

Paywalled Content Pipeline: For papers behind institutional paywalls that cannot be legally downloaded programmatically, we developed a separate ingestion pipeline (`paywalled_pdf_ingestor.py`). This system processes manually obtained PDFs through a PubMed matching agent that: (1) analyzes PDF content to identify title and authors, (2) generates targeted PubMed search queries, (3) validates matches by comparing meta-data, and (4) retrieves complete publication information. Once matched, these PDFs flow through the same extraction pipeline (metadata extraction, ROBIS evaluation, and risk metric extraction) as open-access papers, ensuring consistent data quality across all sources.

This dual-pipeline approach allowed us to incorporate high-quality paywalled meta-analyses while respecting copyright restrictions.

PDF-to-Markdown Conversion: We developed a hybrid extraction pipeline combining Docling (for structural preservation and table extraction) with pymupdf4llm (for character encoding accuracy). The system identifies Unicode encoding errors in Docling output and corrects them using context-based matching with pymupdf’s character-level extraction, preserving document structure while fixing encoding issues.

Quality Assessment: Each paper undergoes ROBIS (Risk Of Bias In Systematic reviews) evaluation (Whiting et al., 2016) via structured LLM prompts. The agent assesses four domains: study eligibility criteria, identification and selection of studies, data collection and study appraisal, and synthesis and findings. This produces both a categorical risk rating and a 0-10 numerical score, enabling downstream filtering by quality thresholds.

Metric Extraction: Risk metrics are extracted via LLM tool calls with strict validation: each metric must include menopause age thresholds (e.g., "≤40 vs 50-55"), specific health outcome, metric type (OR/HR/RR), point estimate, and 95% confidence interval. The LLM is prompted to replace vague age terms like "early menopause" with their numeric definitions used in the meta-analysis, whenever possible, to ensure precise age-risk mappings.

2.4 Data Quality and Output

The pipeline ran for multiple hours, outputting a CSV file, containing 584 risk metrics spanning eight health outcomes: all-cause mortality, type 2 diabetes, cardiovascular disease, all-cause dementia, osteoporosis and fractures, breast cancer, endometrial cancer, and ovarian cancer. Each record includes complete metadata: publication DOI, date, authors, sample size, number of included studies, geographic scope, adjusted confounders, ROBIS scores, and source traceability. This structured dataset enables systematic meta-analysis of menopause timing effects across diverse health outcomes while maintaining full provenance from extraction to analysis.

2.5 Addressing Hallucinations

Addressing hallucinations was one of our main concerns. We implemented several safeguards to ensure the AI agent’s outputs were accurate. The system uses structured tool calls with strict validation schemas that reject incomplete extractions, forcing complete citations and confidence intervals. All extracted metrics retain full source traceability by keeping a DOI reference. We conducted extensive manual spot-checks across disease categories, comparing extracted risk metrics against source tables and forest plots in the original PDFs. Whenever

inconsistencies were found, we searched for their root cause and improved the pipeline so this wouldn't happen again. These measures provide confidence that the extracted metrics accurately reflect the underlying literature.

2.6 Study Selection and Curation

The AI extracted over 250 risk metrics across eight disease categories. To identify the most robust evidence for each disease, we systematically ranked studies according to five criteria: ROBIS quality scores, sample size, publication date, geographic diversity, and confounder adjustment. Studies with low risk of bias and validity scores above 8 out of 10 were given highest priority, as were meta-analyses with large participant numbers and broad study inclusion.

To ensure comparability across diseases, we harmonized risk metrics by selecting studies with similar menopause age group comparisons. Rather than accepting any age threshold, we prioritized results where age cohorts matched closely across all outcomes—for example, "45–50 vs ≥ 51 " wherever possible. Metrics with imprecise or mismatched age groups (e.g., undefined "early vs. normal menopause") were excluded. This approach yielded eight consistent, high-quality meta-analyses—one per disease—with clear, quantitative, age-stratified risk estimates suitable for economic modeling.

3. Estimation Methodology

3.1 Our Approach

From the AI pipeline, we get the risk values (HR, OR, RR) for different diseases and menopause cohorts. For each of eight major health outcomes—all-cause mortality, type 2 diabetes, cardiovascular disease, all-cause dementia, osteoporosis and fractures, breast cancer, endometrial cancer, and ovarian cancer—we pair the extracted risk metric with baseline annual incidence among U.S. women aged 45–64. This cohort represents roughly 41.3 million individuals, capturing both perimenopausal and early postmenopausal women where timing shifts matter most. The net annual change in cases per 100,000 women is then calculated as: baseline incidence $\times$ (RR or HR - 1) for risks, or baseline incidence $\times$ (1 - RR or HR) for benefits.

Once the epidemiologic impact is known, we assign economic weights. Direct costs represent medical expenditures per case—hospitalizations, treatments, medications, and follow-up care. Indirect costs capture lost productivity, informal caregiving, and premature mortality. Some diseases have more precise data and fit the annual costs metric better than others. For example, type 2 diabetes has exact, well-studied yearly direct costs, whereas all-cause mortality does not: the dead have no costs. In such cases, we use the closest reasonable proxy based on the available data. For example, we use end-of-life care for direct medical costs for all-cause mortality, while for indirect costs, we apply the human capital approach (HCA) (Grossman, 2000).

Lastly, we multiply disease-specific cost per case by the net change in cases to obtain the annual economic impact for each condition. Summing across diseases yields the total estimated yearly savings. We then apply the 5-year present value calculation, as therapy would delay the menopause by 5 years. These costs could then be scaled to 130,000 women in US who yearly enter menopause before 45 to find the yearly addressable market. 10% of all women enter menopause prior to 45, for the US population of 171,773,000 women this is the total addressable market of 17,177,000 people.

Below we list every used value for baseline incidence rate, direct cost and indirect costs with sources and reasoning for it, summary tables can also be found in appendix.

3.2 Baseline Incidence Rate

We estimate the baseline annual incidence for U.S. women aged 45–64 using the most recent national datasets and peer-reviewed literature. Each figure represents cases per 100,000 women per year:

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We estimate the baseline annual incidence for U.S. women aged 45–64 using the most recent national datasets and peer-reviewed literature. Each figure represents cases per 100,000 women per year:

- **All-Cause Mortality:** 750 per 100,000. Xu et al., 2022 provides rates by age and sex; we combine 531 for 45–54 and 1,117 for 55–64, adjusting for female-specific patterns and population weights, as women show lower rates than men in the aggregated data.
- **Type 2 Diabetes:** 900 per 100,000. Centers for Disease Control and Prevention, 2023 gives an overall adult rate of 1,010; we adjust for age cohorts and sex, noting women often have lower incidence.
- **Cardiovascular Disease:** 700 per 100,000. Derived from American Heart Association statistics and hospitalization data, this estimate captures events like heart failure around 877, tailored to full CVD in the cohort (Zhang et al., 2023).
- **All-Cause Dementia:** 300 per 100,000. Based on Alzheimer’s Association, 2025, where Alzheimer’s accounts for 60–80% of dementia, we draw from Nature and PubMed studies for a conservative approximation in this age group.
- **Osteoporosis & Fractures:** 200 per 100,000. International Osteoporosis Foundation, 2025 cites national hip fracture incidence of 511–553 overall; we adjust downward for ages 45–64, as most occur in older women.
- **Breast Cancer:** 55 per 100,000. National Cancer Institute, n.d.-a show an all-women rate of 130.8; the 45–64 cohort represents 42% of cases, leading to this scaled estimate.
- **Endometrial Cancer:** 13 per 100,000. National Cancer Institute, n.d.-c indicate an all-women rate of 28.3; the cohort accounts for 45.7% of cases, yielding this figure.
- **Ovarian Cancer:** 4 per 100,000. National Cancer Institute, n.d.-b report an all-women rate of 10; the cohort comprises 40% of new cases, resulting in this low incidence.

These baseline rates anchor our health impact model. When multiplied by the relative risks extracted for menopause cohorts, they produce the annual change in new cases per 100,000 women.

Direct Costs

Direct costs represent average annual medical expenditures per new case of disease. We focus on a per-incident perspective to align with our incidence-based modeling framework. Leaving aside all-cause mortality, cardiovascular disease, all-cause dementia, type 2 diabetes, and osteoporosis & fractures have exact, credible costs. Calculating it for cancers is not as straightforward, as its yearly costs highly vary and are bucketed into three phases: first year, last year before death, and all the years in between. For example, for breast cancer these figures would be: \$33,700, \$3,500, and \$73,200 respectively. What number should one choose for annual costs in such cases? We decided to treat cancers as a 5 year period with 3 continuation years in between. We would sum up these costs and divide by 5 to get a yearly estimate, even though no one single year would have such cost.

- **All-Cause Mortality:** \$80,000. The Arapakis et al., 2022 article serves as a proxy for end-of-life healthcare expenditures, as ongoing costs do not apply post-death.
- **Type 2 Diabetes:** \$12,022. Parker et al., 2024 report provides a precise figure for direct medical spending per patient per year.
- **Cardiovascular Disease:** \$19,145. Shah et al., 2024 details average annual direct medical expenditure per incident case.
- **All-Cause Dementia:** \$53,502. Mehrotra et al., 2025 offers direct healthcare and long-term care costs per case.
- **Osteoporosis & Fractures:** \$50,508. Adeyemi et al., 2019 study covers hospitalization and treatment costs per fracture, a solid fit for incident cases.
- **Breast Cancer:** \$23,480. Mariotto et al., 2020 uses an annualized across-phase estimate, taking the 5 year average with 3 years of continuation.
- **Endometrial Cancer:** \$27,280. Mariotto et al., 2020 uses an annualized across-phase estimate, taking the 5 year average with 3 years of continuation.
- **Ovarian Cancer:** \$44,880. Mariotto et al., 2020 uses an annualized across-phase estimate, taking the 5 year average with 3 years of continuation.

Indirect Costs

Indirect costs capture the broader economic losses beyond healthcare spending, such as lost productivity, absenteeism, and unpaid caregiving. Cancers and fractures have precise

authoritative calculations; the rest of the diseases do not. For them, we look for total annual indirect costs in the US. Dividing it by the total annual number of incidents per disease gives us the indirect cost per incident. We consider these estimates the main risk of our methodology, as one could also argue for calculating them on prevalence-based cases. This would yield meaningfully lower estimates, up to 5-10 times lower per incident. For all-cause mortality, we have used the human capital approach (HCA).

- **All-Cause Mortality:** \$553,800 per premature death. Forbes Advisor, 2024 and Mennini and Gitto, 2022 guide the human capital method: median wage \$55,380 for women aged 45–64 times 10 remaining work years to retirement at 65.
- **Type 2 Diabetes:** \$13,630. Parker et al., 2024 report divides \$106.3 billion in productivity losses for 7.8 annual hospitalization incidents.
- **Cardiovascular Disease:** \$36,500. American Heart Association, 2024 compute indirect costs from \$168 billion total over 4.6 million annual cases.
- **All-Cause Dementia:** \$41,607. USC Schaeffer Center, 2025 study attributes \$233 billion in informal care to 5.6 million cases for this annual per-case value.
- **Osteoporosis & Fractures:** \$5,848. Mariotto et al., 2021 directly reports work absence and reduced activity costs per fracture.
- **Breast Cancer:** \$9,156. American Society of Clinical Oncology, 2024 provides lost productivity per patient per year.
- **Endometrial Cancer:** \$12,500. American Society of Clinical Oncology, 2024 details work and activity impairment per case.
- **Ovarian Cancer:** \$10,000. Lagan et al., 2019 and Cancer Therapy Advisor, 2024 outlines indirect productivity and caregiving losses per case.

The indirect cost estimates complement the direct medical costs, together reflecting the total annual economic burden per disease case.

4. Results

4.1 Health Impacts

The final table with risk values and metrics can be found in appendix. Because the number of accidents per 100,000 people is low, one could use HR and RR interchangeably. OR has to be converted to be used, but in practice it gives similar values. Due to the data constraints, the only sound cohort comparisons we have found for all diseases (but one) was women entering the menopause before and after 45. For some of the diseases we found per-year increase risk values, we scaled them to a 5 year basis. Also in some values we changed the order of comparison (from ≥ 45 vs ≤ 45 to ≤ 45 vs ≥ 45) for convenience of analysis. Osteoporosis and fractures was the only diseases that didn't match this cohort definition, we used the closest available value. Each risk metric has an appropriate paper cited as source.

You can find the net annual change for each disease in appendix, it is derived from applying the risk values to the baseline incidences. Notably there would be a 90 death reductions, 207 less type 2 diabetic cases, 98 less cardiovascular cases. One would get even higher numbers if scaled to 130,000 women that enter the menopause every year in US before 45. Moreover, this impacts spans across a 5-year period given the 5-year therapeutic delay intervention.

4.2 Economic Impacts

With the net annual change for each disease we can also calculate the potential economic savings. When applied, indirect costs of mortality trumpet the other expenditures (resulting in a \$49,842,000) which does make sense: prematurely dead people is a serious loss for society. The second highest cost is the impact of indirect cardiovascular diseases, with a \$3,577,000.

From this data we can calculate the yearly total costs for US. Summing all the direct and indirect costs for 100,000 people and multiplying them by 1.3 to get an estimate for 130,000 women. Lastly, one would need to apply a present value of annuity multiple with a 3% discount rate, a 4.5797 multiple, that results in a \$429,601,314 total yearly cost. One could interpret that number as the yearly addressable market in US. Exact values for each disease can be found in appendix. To find the total addressable market in US we scale these costs to 17,177,300 people: $(17,177,300 / 130,000) * \$429,601,314 = \$56,707,373,000$.

4.3 Limitations

The limitations of our research are plenty. The main issue is the scarcity of high-quality data. First, that led us to focusing on women who enter menopause before and after the age of 45. This is not due to the lack of ambition or desire to study other cohorts, like from 45-50 vs 50+. It is because this was the only cohort that had risk values for all diseases, and thus could be properly modeled. Choosing other cohorts would result in incomplete and invalid comparisons.

Second, that made us have more proxies and approximations for economic data than we would want to. Like calculating the yearly direct costs of cancers that widely differ depending on the year of treatment for patients. Similar issue occurred with indirect costs, where for some diseases we had to approximate the per year per incident costs by dividing the total yearly indirect costs by the total number of hospitalizations. If one would calculate it on prevalence basis yearly costs would be 5-10 times lower. In our defense hospitalizations is what accounts for most costs as they lead to prolonged periods without work and lower productivity in total.

These limitations are serious, hence we suggest the reader to take the final costs as first and foremost *rough* estimates, we recommend considering them within a $\pm 33\%$ range.

As always, more research is needed.

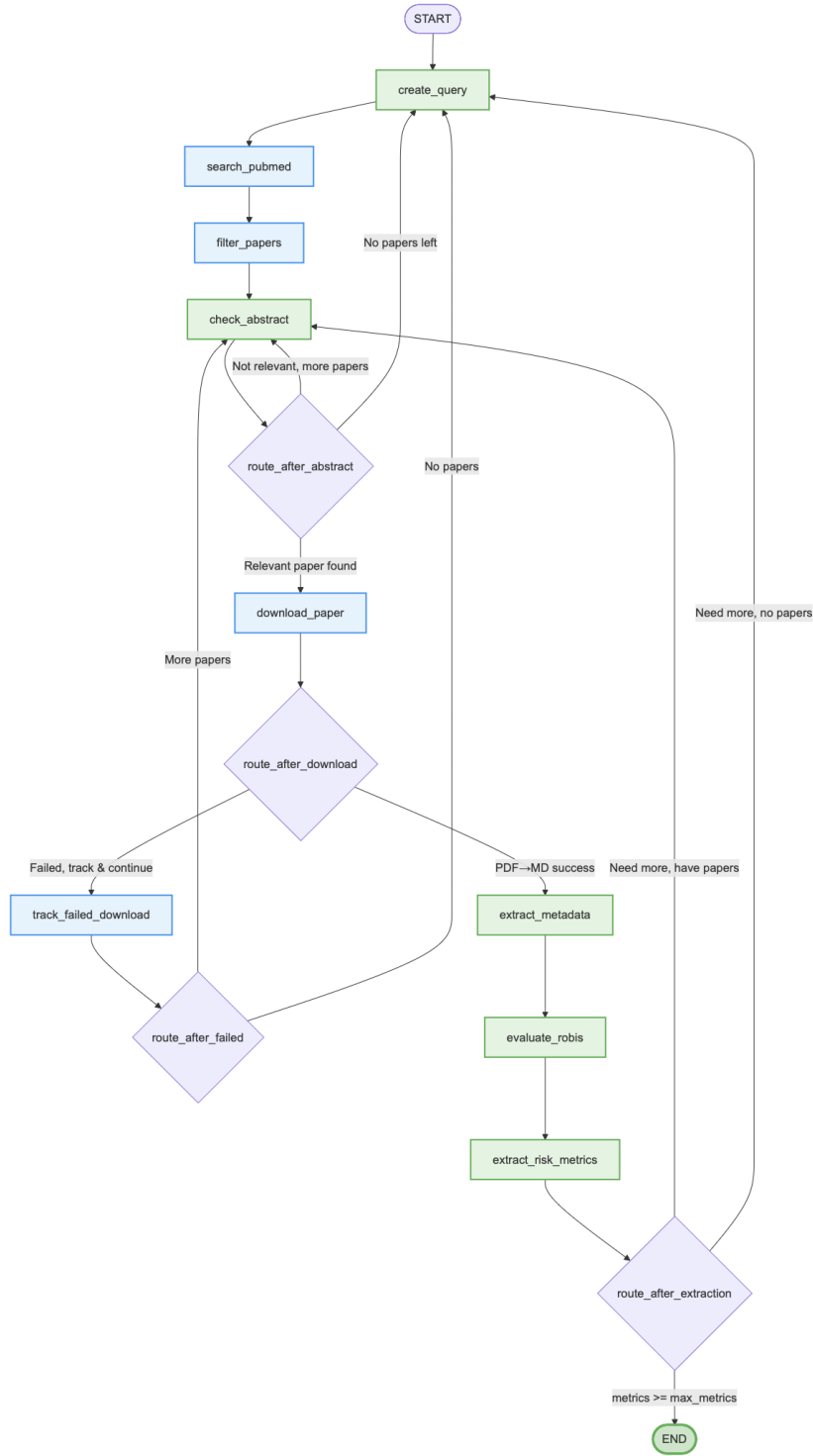


Figure 1: LangGraph workflow architecture for autonomous research paper discovery and risk metric extraction. Green nodes are AI-powered, blue nodes are rule-based, and purple diamonds are routing decision points.

Appendix: Risk Values and Metrics

Disease	Menopause Definition	Risk Metric	Risk Value	95% CI	DOI
All Cause Mortality	<45 vs $\geq$ 45	RR	1.12	1.03 - 1.21	10.1001/jamacardio.2016.2415
Type 2 Diabetes	5-year decrease in ANM	HR	1.230	1.11 - 1.36	10.1007/s00125-017-4346-8
Cardiovascular Disease	40-45 vs >45	RR	1.14	0.97 - 1.35	10.3389/fcvm.2023.1131251
All Cause Dementia	<45 vs $\geq$ 45	RR	1.15	1.03 - 1.28	10.1016/j.eclinm.2021.101236
Osteoporosis & Fractures	45-49 vs >50	RR	1.04	1.03 - 1.05	10.1186/s13018-023-04051-6
Breast Cancer	5-year increase in ANM	RR	0.833	0.787 - 0.885	10.1038/sj.bjc.6604853
Endometrial Cancer	<46.5 vs $\geq$ 46.5	RR	0.962	0.943 - 0.971	10.1155/2019/8584130
Ovarian Cancer	5-year increase in ANM	RR	0.734	0.624 - 0.859	10.3389/fonc.2025.1541585

Table 1: Risk metrics extracted from meta-analyses for each disease category. ANM = Age at Natural Menopause, RR = Relative Risk, HR = Hazard Ratio, CI = 95% Confidence Interval, Source = DOI.

Appendix: Annual Incidences and Net Effects

Disease	Annual Incidences	Net Annual Change
All Cause Mortality	750	90
Type 2 Diabetes	900	207
Cardiovascular Disease	700	98
All Cause Dementia	300	45
Osteoporosis & Fractures	200	8
Breast Cancer	55	-9.185
Endometrial Cancer	13	-0.494
Ovarian Cancer	4	-1.064

Table 2: Annual incidences per 100,000 women aged 45-64 and net annual change in cases per 100,000 women.

Appendix: Annual Direct and Indirect Costs

Disease	Annual Direct Costs	Annual Indirect Costs
All Cause Mortality	\$80,000.00	\$553,800.00
Type 2 Diabetes	\$12,022.00	\$13,630.00
Cardiovascular Disease	\$19,145.00	\$36,500.00
All Cause Dementia	\$53,502.00	\$41,607.00
Osteoporosis & Fractures	\$50,508.00	\$5,848.00
Breast Cancer	\$23,480.00	\$9,156.00
Endometrial Cancer	\$27,280.00	\$12,500.00
Ovarian Cancer	\$44,880.00	\$10,000.00

Table 3: Annual direct medical costs and indirect costs (lost productivity, caregiving) per case for each disease category.

Appendix: Annual Costs Scaled to 100,000 Women

Disease	Net Annual Change	Annual Direct Costs	Annual Indirect Costs	Direct Cost Change per 100k	Indirect Cost Change per 100k	Total Cost Change per 100k
All Cause Mortality	90	\$80,000	\$553,800	\$7,200,000	\$49,842,000	\$57,042,000
Type 2 Diabetes	207	\$12,022	\$13,630	\$2,488,554	\$2,821,410	\$5,309,964
Cardiovascular Disease	98	\$19,145	\$36,500	\$1,876,210	\$3,577,000	\$5,453,210
All Cause Dementia	45	\$53,502	\$41,607	\$2,407,590	\$1,872,315	\$4,279,905
Osteoporosis & Fractures	8	\$50,508	\$5,848	\$404,064	\$46,784	\$450,848
Breast Cancer	-9.185	\$23,480	\$9,156	-\$215,664	-\$84,098	-\$299,762
Endometrial Cancer	-0.494	\$27,280	\$12,500	-\$13,476	-\$6,175	-\$19,651
Ovarian Cancer	-1.064	\$44,880	\$10,000	-\$47,752	-\$10,640	-\$58,392

Table 4: Economic impact of 5-year therapeutic menopause delay per 100,000 women. Net annual change multiplied by per-case costs yields total cost changes.

Appendix: Annual Costs Scaled to 130,000 Individuals

Disease	Direct Cost Change per 130,000	Indirect Cost Change per 130,000	Total Costs per 130,000	Multiplier Ap- plied
All Cause Mortality	\$9,360,000.00	\$64,794,600.00	\$74,154,600.00	\$339,605,821.62
Type 2 Diabetes	\$3,235,120.20	\$3,667,833.00	\$6,902,953.20	\$31,613,454.77
Cardiovascular Dis- ease	\$2,439,073.00	\$4,650,100.00	\$7,089,173.00	\$32,466,285.59
All Cause Dementia	\$3,129,867.00	\$2,434,009.50	\$5,563,876.50	\$25,480,885.21
Osteoporosis & Frac- tures	\$525,283.20	\$60,819.20	\$586,102.40	\$2,684,173.16
Breast Cancer	-\$280,362.94	-\$109,327.22	-\$389,690.16	-\$1,784,664.02
Endometrial Cancer	-\$17,519.22	-\$8,027.50	-\$25,546.72	-\$116,996.30
Ovarian Cancer	-\$62,078.02	-\$13,832.00	-\$75,910.02	-\$347,645.10

Table 5: Economic impact per 130,000 individuals across disease categories.

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